
PREPARED FOR CARBON

Hospital receivables, verified, recovered and funded

A partnership in which Carbon lends against claims the payors owe, and we make sure they pay.

PREPARED FOR

Chijioke Dozie, Carbon

PREPARED BY

Dr Debo Odulana

MEETING

8 October 2026



Carbon lends against claims the payors owe. We make sure the payors pay.

60%

of a verified claim's value paid to the hospital within 48 hours

8

checks every claim passes before any money moves, and a person signs each one

799

Lagos providers already in our directory, with a direct line to 702 of them

Live

public page, claims desk and decision rules are running in production today

WHAT CARBON DOES

- Lends to the hospital against its verified claims, through a collection account the payors pay into.
- Sets the credit policy and the facility per hospital, and takes a one time discount of 4% to 5% of the advance at disbursement.
- Receives claim-level performance data, monthly or live.

WHAT WE DO

- Find the hospitals, verify every claim and keep the bad ones out of the book.
- Settle disputes with the payors, chase, escalate and collect, for a one time fee of 1.2% of the advance, also taken at disbursement.
- Never lend, never buy a claim, never guarantee. Factoring needs a licence we do not hold and do not want.

For tomorrow: agree the route (lend against the claims or buy them), the pilot size, the discount within its range and the data Carbon needs to underwrite.



Private hospitals are financing the payors.

Most of what a private hospital earns from insured patients is owed by HMOs, with corporate accounts and state schemes behind them. The work is done and the cost is paid this month. The cash arrives months later, if the claim survives the payor's review at all.

56 days

average time to process a claim on the largest Nigerian HMO claims platform in 2025, down from 81 days in 2024

8.4m

claims a year through that one platform, worth about N22bn, across 20 to 25 HMOs

30 days

the most the Guild of Medical Directors said in 2022 its members would tolerate being owed, with interest after

"They have not paid us in three months after treating more than 250 patients."

A Lagos private facility manager, in a study of why facilities hesitate to join insurance schemes

Sources: Medismarts figures as reported by Streamlinefeed, July 2026 (vendor-reported; the article does not say whether this is submission to payment or to adjudication). Guild of Medical Directors, BusinessDay and Daily Trust, January 2022. Facility manager: PLOS ONE, 2021, fieldwork in Lagos 2017 to 2018. We found no published Nigerian rejection rate. The pilot will produce the first one.



Most of it is disputed. Very little of it is refused.

Why the claim is unpaid	What clears it	Recoverable
No pre-authorisation code, or the code does not match the service billed	Retrospective authorisation, agreed with the payor's medical team	Usually
Billed above the agreed tariff, or on an old tariff	Rebilled at the agreed rate; the gap is written off, not the claim	Mostly
Missing documents: notes, results, discharge summary	Pull the record and resubmit inside the payor's window	Usually
Submitted late, or never submitted	Depends on the payor's window and goodwill	Sometimes
The payor is slow to pay across the board	Escalation, a payment schedule, then the regulator	Slowly
The enrollee was not covered, or the payor has failed	Bill the patient or employer, or write it off	Rarely

This is why the risk is better than it looks from outside. A claim that has been verified is a debt from a regulated payer for work already done, and most of what blocks payment is paperwork that can be fixed before Carbon lends a naira.



The law, the rate cycle and the failures all point the same way.

Late payment is an offence

The NHIA Act 2022 makes it an offence for an HMO to fail to pay providers or settle claims within the period in the operational guidelines (s.48), and sends disputes to NHIA mediation first (s.47). Recovery has a regulator behind it.

Receivables are assignable

The Secured Transactions in Movable Assets Act 2017 makes an assignment of a receivable effective against the debtor even where the contract limits assignment (s.4(2)), with priority by registration at the National Collateral Registry (s.33).

A Factoring Act is close

The Factoring, Assignments and Receivables Financing Bill 2026 passed the National Assembly in June 2026 and went for assent. We could not confirm it has been signed.

Unsecured SME credit has failed

Lidya closed in October 2025 on loan losses and naira depreciation. Lending repaid from a named, regulated payer's cash is a different risk, which is the case for this book.

Sources: NHIA Act 2022 (Gazette No. 95, May 2022); STMA 2017; Senate proceedings and Afreximbank on the Factoring Bill, June 2026; TechNext, October 2025. The CBN also cut the policy rate from 26.5% to 23% on 22 September 2026, though lenders warn the cost of credit will lag.



Verify, reconcile, recover

One desk, run by people who have run hospitals and sat across the table from payors, on a platform we built for the job.

Verify

Every claim is read against the payor's tariff, the authorisation record, the documents held and its age. The platform drafts the verdict, the likelihood of payment and the exact repairs; a person confirms it. Claims that will not pay are kept out of the funded book and sent to repair.

Reconcile

We match what the hospital billed against what each payor says it owes, then clear the gap claim by claim with the payor's claims team. For each payor the platform ranks the claims by naira recovered per hour of effort and drafts the reconciliation letter and the call script.

Recover

We chase, escalate and settle. Every call is logged and the platform turns the notes into dated promises, disputes and next actions, so a promise made on a Tuesday is chased on the Friday. Payment lands in the account Carbon controls.



Sixty per cent within 48 hours, priced once, the balance when the payor pays.



ONE CLAIM, END TO END (ILLUSTRATIVE)

	Naira	When
Billed to the payor	1,000,000	Day 0
Verified payable (after tariff correction)	950,000	Day 1
Advance, 60% of verified	570,000	By day 3
Less Carbon's one time discount, 4% to 5%	22,800 to 28,500	At disbursement
Less CFA's one time fee, 1.2%	6,840	At disbursement
Hospital receives now	534,660 to 540,360	By day 3
Payor pays into the collection account; Carbon takes back 570,000	950,000	Day 30 to 90
Hospital receives the balance	380,000	On receipt



Every advance passes the same eight checks, in this order.

#	Check	If it fails
1	Funding is live with a signed funder, and the hospital's agreement is signed and active	Hold
2	The claim has been verified, and a person has confirmed the verification	Hold
3	Verdict is PASS. A claim that needs repair is repaired first; one that will not pay is declined	Hold or decline
4	Likelihood the payor pays is at least 75%	Hold
5	The payor is eligible, and is not one CFA advises	Decline
6	Claim age is within the limit (180 days by default, set per hospital)	Decline
7	The advance keeps the hospital inside its facility limit	Hold
8	No single payor above 40% of the hospital's advanced book, once the book is past a quarter of its limit	Hold

The amount

60% of the lowest of what was billed, the agreed tariff and the verified payable. Rounded down to the kobo.

Who decides

The rules decide. The model's view can stop an advance but never authorise one. A person approves every advance, and Carbon's own credit decision comes after that.



What the desk sees before anyone lends.

Two test claims run through the platform this week. The output below is what it returned, lightly shortened.

Caesarean section, 3 nights

Payable after rebill

Billed N450,000 Tariff N420,000 Authorised 117 days

FOUND

- Authorisation, documents and timing are in order
- Only query: N30,000 billed above the agreed tariff

REPAIRS, IN ORDER

- Confirm N420,000 is the current tariff
- Reissue at N420,000 under the same authorisation code
- Include the operation note with the clinical notes
- Reply to the tariff query in writing and ask for a payment date

Expected payable N420,000

Appendicectomy

FAIL, 12%

Billed N600,000 No tariff on file No authorisation 232 days

FOUND

- Never submitted, 232 days after service
- No authorisation for a procedure that normally needs one
- No documents held, cover not confirmed

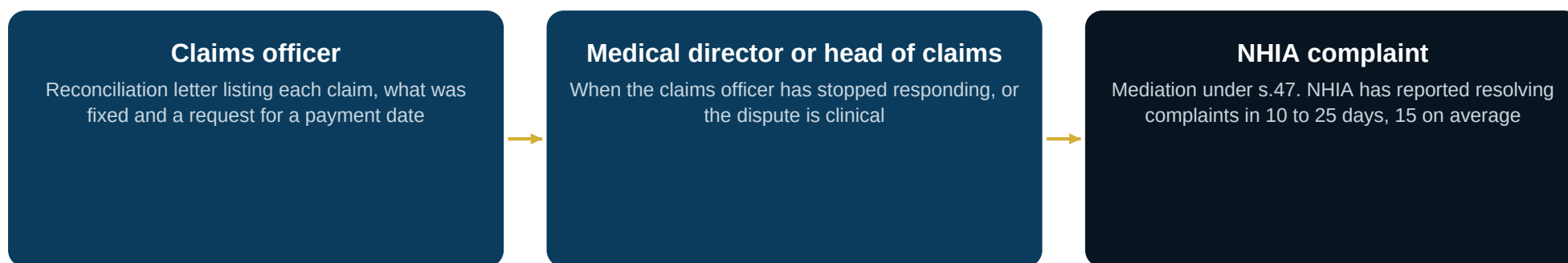
REPAIRS, IN ORDER

- Establish whether it was an emergency and whether the payor was told
- Check the submission window and tariff in the provider agreement
- Only then decide whether to pursue retrospective authorisation

Kept out of the funded book



Reconciliation first, the regulator last.



WHAT THE PLATFORM DOES ON EVERY CALL

- Reads the call notes and records each promise with its amount, its claims and its date
- Logs each dispute against the claim it concerns
- Sets the next action and its due date, so nothing promised is left unchased

FROM A TEST CALL THIS WEEK

"She accepts CL-001 at the tariff of 420k, in the next payment run, end of October. CL-002 has no PA code, so they will not pay unless the medical director approves a retrospective authorisation."

Recorded as: a promise of N420,000 on CL-001 by 31 October; a dispute on CL-002; six follow ups, the first due tomorrow.



Running in production since 7 October

CHECK YOUR RECEIVABLES

What is sitting outside your hospital?

YOUR NUMBERS

Billed to HMOs and corporates each month **₦15.0m**

Average days until a claim is paid **120 days**

Share of claims queried, cut or rejected **20%**

₦60.0m
owed to you at any one time

₦12.0m
of that, held up in queries and disputes

₦15.0m
released for good if you are paid 30 days sooner

This is arithmetic on the figures you enter, not a forecast of what we will recover. The written read is.

GET THE WRITTEN READ

Free. We review 20 of your unpaid claims and tell you what is recoverable, from whom, and how fast.

Hospital or clinic

Your name Your role

Email Phone (WhatsApp)

City How many HMOs pay you?

Oldest unpaid claim

Anything we should know (optional)

☐ I am interested in early payment against vetted claims when it opens

☐ Consult for Africa may hold these details to reply to me about this service. We never ask for patient names or records at this stage.

Send me the written read →

Public page

A hospital enters its billing, days to payment and query rate, sees what is sitting outside the business, and asks for a free review of 20 claims.

Directory and funnel

Every hospital, every contact and every stage from target to signed, with call and WhatsApp logging.

The desk

Claims received from a spreadsheet, verified, decided, approved, advanced, chased and settled, with an append-only record of every step.

Data for Carbon

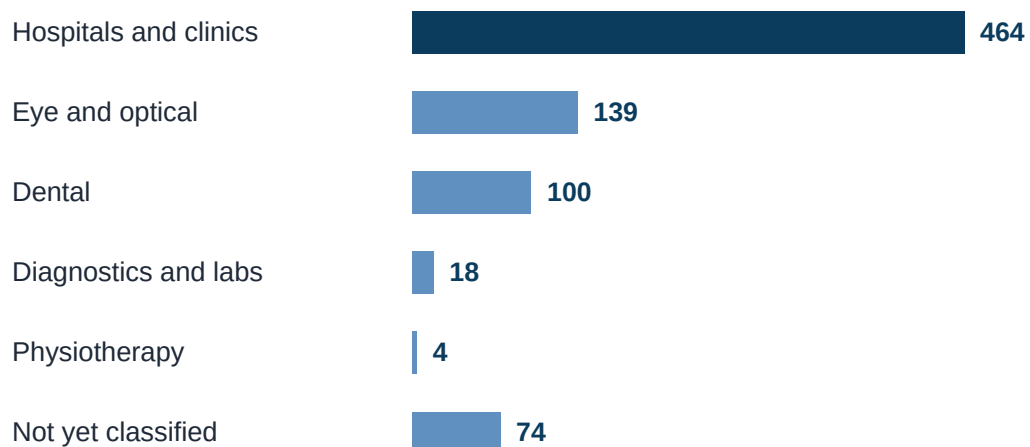
Claim-level status, promises and payments, ready to share as a feed or a monthly file.

consultforafrica.com/services/claims-recovery, live since 7 October 2026: the receivables check and the free review offer



Where the first book comes from

799 LAGOS PROVIDERS IN OUR DIRECTORY, BY TYPE

**702**

with a working email on file

698

with a direct phone number

21

local government areas covered

190

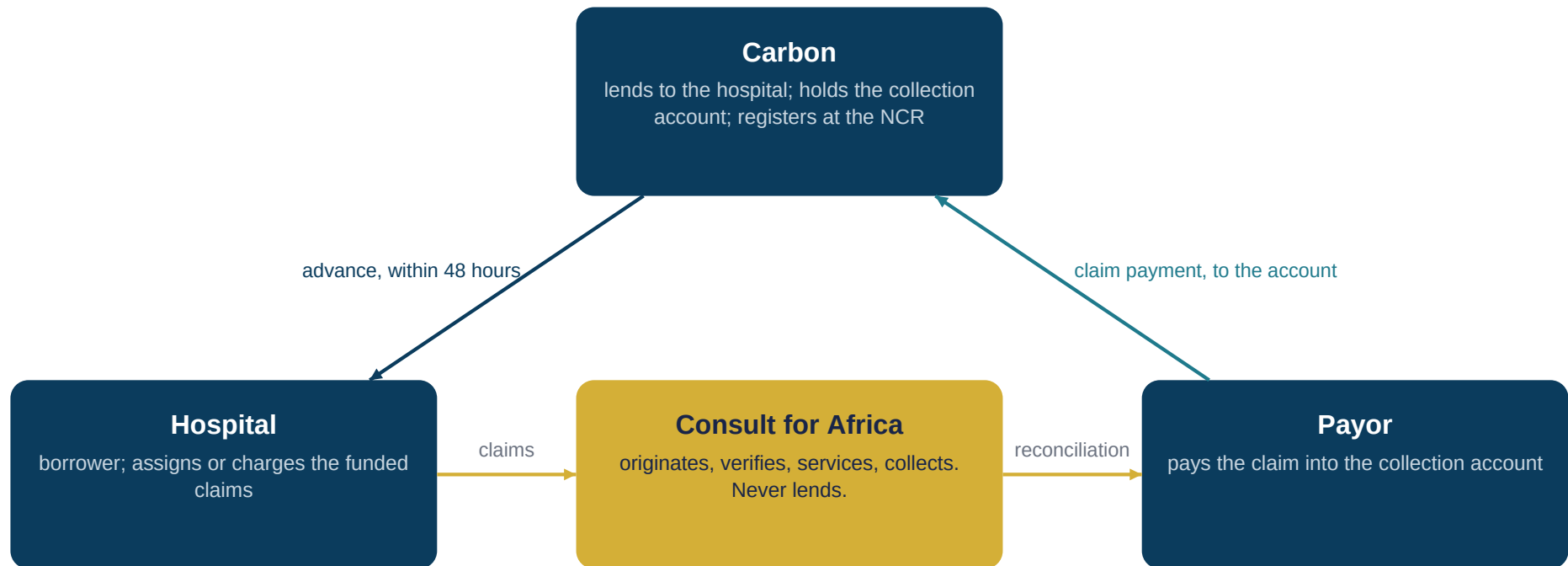
in Eti-Osa alone, the densest private market

HOW WE REACH THEM

- Direct: a call and a WhatsApp from a named person, then the free review of 20 claims. The review is the conversion step, and it costs the hospital nothing.
- Through the profession: the Guild of Medical Directors and the private practitioners' associations have campaigned on HMO debts since at least 2022. One endorsed member offer is worth more than the whole cold list.
- Through our own work: hospitals whose operations and accounts we already run or have audited.



How Carbon gets to the cash



- Collection account at Carbon with a domiciliation and lien; the payor is told to pay there.
- Register every funded claim at the National Collateral Registry. Priority goes by time of registration (STMA s.33(3)).
- Serve notice of assignment on a payor only on default, slow payment or a set-off risk, so Carbon stays out of the hospital's relationship with the payor.
- Vet for open clawbacks: set-off the payor acquires before notice binds the lender (s.33(1)(b)).

For Carbon's counsel to confirm: whether Carbon lends against the claims or buys them outright, and the notice and registration mechanics above.



The division of labour, line by line

Carbon	Consult for Africa	The hospital
Sets the credit policy and the facility limit for each hospital	Brings the hospitals, verifies every claim before it is funded	Assigns or charges the funded claims, and tells the payors where to pay
Advances 60% of verified value within 48 hours	Reconciles and recovers with the payors	Provides records, and repairs claims we send back
Holds the collection account the payors pay into	Reports every claim's status to Carbon and the hospital	Repays any advance on a claim rejected for a reason it controlled
Takes its one time discount of 4% to 5% at disbursement	Takes its one time fee of 1.2% at disbursement, and keeps the claims data clean enough to lend on	Submits its whole book with each payor, not only its worst claims
Registers at the NCR and holds the security	Runs the conflicts policy for any payor it advises	Keeps submitting through the desk, not around it



Priced once, at disbursement, so the payor's speed is Carbon's yield.

4% to 5%

Carbon's discount on the advance, one time, taken at disbursement

1.2%

Consult for Africa's fee on the advance, one time, taken at disbursement

5.2% to 6.2%

all in for the hospital, known before it accepts, and nothing more however long the payor takes

CARBON'S ANNUALISED RETURN ON THE ONE TIME DISCOUNT, BY DAYS UNTIL THE PAYOR SETTLES

Discount	45 days	60 days	90 days	120 days
4.0%	32.4%	24.3%	16.2%	12.2%
4.5%	36.5%	27.4%	18.3%	13.7%
5.0%	40.6%	30.4%	20.3%	15.2%

For reference: bank invoice discounting (FirstBank) 32% a year plus a 1% fee; the CBN policy rate 23% since 22 September 2026.

This is why recovery and funding belong in one product. Every week we take off a payor's settlement raises Carbon's return without costing the hospital a naira more, and the payors that pay slowly are exactly the ones the pilot will find and price.



Nobody in Nigeria joins verification, recovery and funding.

Who	What they do	What it tells us
Curacel	Claims processing and fraud detection for insurers and HMOs, linking 800+ hospitals. Announced provider cash advances in 2021	The rails exist; we could not confirm the advance product is live
Medismarts	Claims platform used by 20 to 25 HMOs, about 8.4m claims a year	Sits on the HMO side, not the hospital's
Medical Credit Fund and uniBank (Ghana, 2016)	NHIS claims pre-financing: eligible claims assigned to the bank, a discounted amount paid out	The closest precedent in West Africa. MCF reports 94% to 96% repayment across its health SME book
Generic SME lenders	Unsecured working capital	Lidya's closure in 2025 is the cautionary tale

The gap is a lender that can trust what it is funding. That trust comes from verifying the claim and owning the recovery, which is the part we do.



What could go wrong, and what stops it

Risk	What it looks like	Control
Adverse selection	Hospitals send their weakest claims for funding	Whole-book submission per payor to qualify; only verified PASS claims funded
Fraud and duplicates	Padded or invented claims; the same claim funded twice	Duplicate checks on payer, amount and date; NCR search before funding; hospital warranty with recourse
Payor failure	A payor stops paying or loses accreditation	Per-payor limits and the 40% concentration cap; lower or no advance on weak payors
Set-off	A payor deducts an old overpayment from a funded claim	Vet for open clawbacks; serve notice as soon as a set-off risk appears
Payor pushback	A payor slows authorisations for hospitals that use the desk	Reconciliation first; notices only when needed; the regulator only after the relationship route
Conflict of interest	We recover from a payer CFA also advises	Written policy: that payer is never advanced against, and both sides are told in writing
Data protection	Health data leaves the hospital	No patient names held or processed; claim references only; a data protection impact assessment before the pilot



Ninety days, eight to twelve hospitals, four numbers.

8 to 12

private hospitals in Lagos

N100m to 200m

of verified claims

90 days

from first batch to review

4

numbers that decide what happens next

Measure	What it tells Carbon
First-pass acceptance	Share of verified claims the payor pays without query. The quality of our verification
Days to cash	Submission to payment into the collection account. How long Carbon's money is out
Recovery rate	Share of disputed value recovered. The quality of our recovery
Loss rate	Advances not repaid by the payor or the hospital. The number that sets the price

Weeks 0 to 2

Terms, counsel, collection accounts, data agreement

Weeks 2 to 4

Free reviews; first hospitals signed; first batches verified

Weeks 4 to 12

Funding cycles; weekly shared dashboard

Day 90

Decide rate, recourse and facility size on our own numbers

Stop-loss and step-up triggers are agreed before the first advance: the loss rate that ends the pilot, and the payment record that lifts a payor's advance rate.



Three lines that follow once the claims history exists

Surgery finance for patients

Our doctor network sees patients at the point where surgery is recommended and priced. The money goes straight to the facility, the surgeon confirms the procedure and the price is a fixed package, which is how this kind of lending keeps defaults low. Carbon's interest, and a natural second line.

Doctors' pending claims

Specialists in private practice wait on the same payors as the hospitals do. The same verification and the same advance, sized for an individual practice.

Claims submission, done right first time

Once a hospital's backlog is cleared, we take over its monthly claims preparation, so fewer claims are queried in the first place and the funded book gets cleaner every month.

The order matters. Recovery builds a claim-level payment history for every hospital and every payor, and that history is what lets Carbon price each of the next three lines with evidence rather than hope.

Four things to settle, and then a first batch

- **The route.** Carbon lends against the claims, or buys them. Counsel on both sides to confirm.
- **The pilot.** Facility size, the hospitals, and the stop-loss and step-up triggers.
- **The price.** 4% to 5% to Carbon and 1.2% to us, one time at disbursement. Where in the range, and what moves it.
- **The data.** What Carbon needs to underwrite, in what form, and how often.

Then, within a month: signed pilot terms, the first hospitals reviewed, and a first verified batch ready to fund.

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